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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°156 /MVE-BDBV/17/10/2026

Report date: October 17, 2026 Publication date: October 18, 2026

HIGHLIGHTS (last 24 hours)

- During the day of October 17, 2026, forty (40) new confirmed cases of Ebola Virus Disease (EVD), including 9 community deaths, were reported. These cases originated from the provinces of Ituri (27 cases), North Kivu (9 cases), Haut-Uélé (2 cases), and Tshopo (2 cases). In addition, 16 deaths were recorded in Ebola Treatment Centers (ETCs). No new health zones were affected in the last 24 hours.
- A total of 411 new samples were analyzed; 40 of these samples tested positive for virusBundibugyo, representing a 9.7% positivity rate

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9455	4,561	48.2%	925	2,530	80.0%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé and South Ubangi

66 Health Zones affected out of 178 (37.1%)

Summary of key points from the last 24 hours

Over the past 24 hours, 40 new confirmed cases of Ebola virus disease (EVD) have been reported in four provinces: **Ituri (27 cases),** North Kivu **(9 cases), Haut-Uélé (2 cases), and Tshopo (2 cases).** Among these cases, **9 community deaths** have been reported. In addition, **16 confirmed deaths** have been recorded in Ebola Treatment Centers (ETCs), **bringing the total number of confirmed deaths to 25.**

notified during the period

In addition, **8 patients (7 in Ituri and 1 in North Kivu)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded **9,455 confirmed cases and 4,561 deaths, representing a case fatality rate of 48.2%.** Ituri remains the epicenter of the epidemic, accounting for **73.2%** of cumulative confirmed cases and **67.5%** of new confirmed cases reported in the last 24 hours.

66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 17, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	27	6918	3,197	46.2%	28/36 (77.8%)
North Kivu	9	2036	1,156	56.8%	17/34 (50.0%)
Haut-Uélé	2	408	176	43.1%	7/13 (53.8%)
Tshopo	2	73	25	34.2%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	2	1	50.0%	1/11 (9.1%)
Total	**40**	**9,455**	**4,561**	**48.2%**	**66/178 (37.1%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 17, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6918**	**3197**	**46.2%**	**27**	**5**	**12**	**17**
Adja	11	1	9.1%				0
Aru	11	6	54.5%	1	0	0	0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	216	48	22.2%				0
Boga	2	2	100.0%				0
Bunia	1881	628	33.4%	4	2	1	3
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	188	113	60.1%				0
Lita	309	156	50.5%	1	0	0	0
Logo	14	9	64.3%	0	0	1	1
Lolwa	36	9	25.0%				0
Mahagi	4	1	25.0%				0
Mambasa	68	28	41.2%				0
Mandima	330	165	50.0%	14	2	7	9
Mangala	379	162	42.7%				0
Mongbwalu	713	342	48.0%				0
Nia-Nia	330	191	57.9%	1	1	0	1
Nizi	827	298	36.0%	3	0	0	0
Nyankunde	134	40	29.9%			1	1
Rimba	19	5	26.3%				0

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Rwampara	1160	402	34.7%	3	0	2	2
Tchomia	76	25	32.9%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2036**	**1156**	**56.8%**	**9**	**4**	**2**	**6**
Blessed	506	341	67.4%	4	2		2
Good	23	7	30.4%				0
Butembo	311	187	60.1%				0
Goma	1	0	0.0%				0
Kalunguta	76	29	38.2%				0
Katwa	639	392	61.3%	0	0	1	1
Kayna	3	2	66.7%				0
Kyondo	44	21	47.7%				0
Lubero	35	14	40.0%				0
Mabalako	74	30	40.5%	4	2		2
Alimbongo	5	3	60.0%				0
Manguredjipa	15	5	33.3%				0
Masereka	39	13	33.3%				0
Musienene	151	71	47.0%				0
Mutwanga	24	5	20.8%				0
Oicha	50	22	44.0%	1	0	1	1
Vuhovi	40	14	35.0%				0
Upper Uele	**408**	**176**	**43.1%**	**2**	**0**	**2**	**2**
Boma Mangbetu	45	19	42.2%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	106	46	43.4%	1	0	0	0
Pawa	102	55	53.9%	0	0	2	2
Rungu	5	3	60.0%				0
Wamba	146	51	34.9%	1	0	0	0
Tshopo	**73**	**25**	**34.2%**	2	0	0	0
Bafwasende	17	2	11.8%				0
Kabondo	9	1	11.1%	1	0	0	0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	33	14	42.4%	1	0	0	0
Mangobo	10	7	70.0%				0
Tshopo	2	0	0.0%				0
Wanie -Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Wapinda	2	1	50.0%				0
Total	**9455**	**4561**	**48.2%**	**40**	**9**	**16**	**25**

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*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified as of October 17, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	125	8	133	104	8	21	0	112	1
Bas Uélé	130	0	130	18	0	112	0	18	0
Ituri	1,347	73	1,420	119	71	1,220	2	182	61
North Kivu	793	29	822	99	29	694	0	128	86
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	153	1	154	9	1	144	0	10	3
Total General	**2,548**	**111**	**2,659**	**349**	**109**	**2,191**	**2**	**450**	**151**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

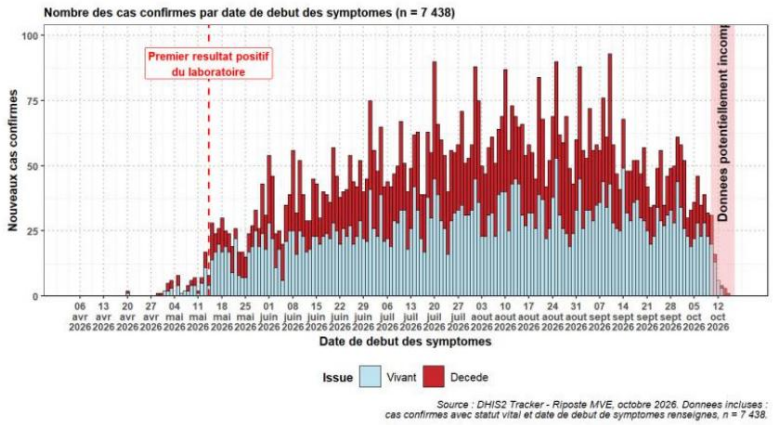
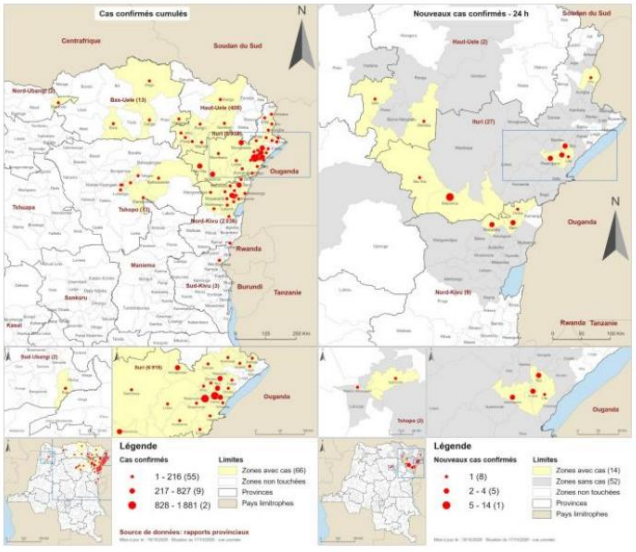


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



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1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

- During the day of October 17, 2026, 2,659 alerts were reported in the five provinces that submitted their data: Ituri, North Kivu, Haut-Uélé, Bas-Uélé, and Tshopo. Of these alerts, 2,651 (99.7%) were verified, with 458 (17.3%) validated as suspected cases and 2,193 (82.7%) invalidated. Of the 458 validated suspected cases, 450 (98.3%) were investigated, with 151 (33.6%) transferred to COVID-19 treatment centers.

- Of the 35,162 contacts to be followed up in the six provinces that submitted their data, 28,144 were seen in the last 24 hours, representing a follow-up rate of 80.0%, slightly higher than the previous day (79.1%) but below the required threshold of 95.0%. This rate was 100.0% (44/44) in South Ubangi, 93.0% (749/805) in Bas-Uélé, 90.4% (1,358/1,503) in Haut-Uélé, 82.8% (995/1,202) in Tshopo, 81.3% (14,563/17,923) in North Kivu and 76.3% (10,435/13,685) in Ituri, where monitoring remains particularly low in Bunia (27.7%; 1,004/3,620) and in Nia-Nia (57.1%; 220/385); In North Kivu, it is only 0.0% (0/215) in Manguredjipa, 53.0% (157/296) in Kyondo and 58.5% (741/1267) in Mabalako, and in Haut-Uélé it is 79.3% (130/164).

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

- On October 17, 2026, 5 alerts were notified to the PoE/PoC in the five monitored provinces: 2 in Ituri (PoC DGRPI Nizi and Biakato Mayi), 1 in North Kivu (PoC Foner Kasindi), 1 in South Kivu (PoE of Kavimvira), and 1 in Haut-Uélé (PoC Mutowamba). These included 3 live alerts validated as suspected cases (2 in Ituri and 1 in North Kivu), of which 2 (66.7%) were transferred to CT/CTE or referred to a healthcare facility, the suspected case in Biakato Mayi having refused transfer, 1 lifeless body intercepted and swabed at the PoE in Kavimvira (South Kivu), and 1 alert invalidated after review in Haut-Uélé.

Table 4. Status of key PoE/PoC indicators by province, as of October 17, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	98.4% (60/61)	95.2% (20/21)	55.6% (5/9)	100.0% (18/18)	90.0% (9/10)	94.1% (112/119)
% of functional strategic PoE/PoC	100%	100%	ND	100%	90%	ND
Number of people switched to PoE/PoC	138,695	91,827	7,760	20,297	24,599	283,178
% of travelers screened	97.3%	97.9%	98.6%	98.9%	99.5%	97.8%
% of travelers who washed their hands	96.9%	97.9%	98.6%	98.5%	99.5%	97.6%
% of travelers made aware	99.6%	97.9%	100.0%	100.0%	100.0%	99.1%
Number of alerts notified	2	1	1	0	1	5
Number of live alerts validated (suspected cases)	2	1	0	0	0	3
Number of bodies recovered today	0	0	1	0	0	1
% of suspected cases transferred to the CTE/CT	50.0%	100.0%	N / A	N / A	N / A	66.7%
% of lifeless bodies swabed/secured	N / A	N / A	100.0%	N / A	N / A	100.0%
% of suspected cases investigated within 4 hours	100.0%	100.0%	N / A	N / A	N / A	100.0%

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2.2.2. Challenges

- **In Ituri,** insufficient mobility for supervision in the health zones of Rwampara, Nizi, and Bunia, delayed payments to service providers for September, and the need for an ambulance to serve the points of presence (PoPs) in the Mandima health zone persist; the Avakubi PoP remains non-functional due to insecurity. In North Kivu, the coverage rate has risen to 95.2% (20/21), but the Butuhe PoP remains closed since the security incident of September 26, 2026; delayed response to alerts from PoPs, security threats against service providers along the Butembo axis, lack of communication resources, shortages of consumables (infection prevention supplies, personal protective equipment, monitoring tools), and the absence of an ambulance dedicated to PoPs are reported. In South Kivu, completeness remains low (55.6%), and the Kavimvira, Kalundu Port SNCC, and Baraka PoEs lack isolation devices. In Haut-Uélé, the Matari PoC did not report any revenue due to a lack of flight scheduling. In Bas-Uélé, PNHF services are not operational, service providers are demotivated and untrained, and water supply, mobility, connectivity, visibility badges, and PoE/PoC digitization are lacking.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 27 new positive results** (22 living and 5 deaths) out of 165 samples received and analyzed **(positivity: 16.4%).**
- **North Kivu: 9 new positive results** (5 living and 4 dead) out of 185 samples received and analyzed **(positivity: 4.9%).**
- **Haut-Uélé: 2 new positive results** (2 living and 0 deaths) **out of 30 samples received and analyzed (positivity: 6.7%).**
- **Tshopo: 2 new positive results** (2 living and 0 deaths) **out of 12 samples received and analyzed (positivity: 16.7%).**
- **Bas Uélé:** 19 new samples received at the Buta and Isiro laboratory, all analyzed negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 27 out of 35 rings were opened (77.1%), 45 out of 58 households were decontaminated (77.6%), and 4 out of 74 FOSA (Health Facilities) were decontaminated out of 5 (80.0%). Out of 89 EDS alerts, 79 (82.3%). ，used for blood and sanitation assessments (83.1%) and 65 bodies were swabed.

- **Tshopo :** 10 ESS were followed in 4 ZS and 169 providers were briefed on standard precautions, triage and screening; the IPC score of CS Mbilinga was reassessed at 43.5%, CS Kumbasika was supplied with IPC inputs, the PPLs of the Rekapi Polyclinic were assessed around a confirmed case (5 at high risk and 10 at intermediate risk) and the household of the 64th confirmed case was decontaminated; 2 EDS were missed at Rekapi 1.

- **Haut-Uélé:** 1 out of 1 infection control zones (100%) have been opened, 3 out of 3 households and 2 out of 2 primary healthcare facilities (PHFs) have been decontaminated in Pawa, including the Libiki clinic, and the Pawa infection treatment center (ITC) has been decontaminated around confirmed cases; 232 students were briefed on hand hygiene at the Mgr Olombe school during International Hand Hygiene Day, and the infection prevention and control (IPC) scores of 15 Pawa PHFs, reassessed, range from 20% at the La Providence health center to 80% at the Nebobongo hospital. Of 2 COVID-19 alerts, 1 IHF was carried out and 1 body was swabbed in the Abiangama health district, the other is underway in Gatoa.

- **North Kivu:** 22 rings were opened out of 31 (71.0%), 10 households decontaminated out of 20 (50.0%) and 14 ESS out of 17 (82.4%); 10 ESS were assessed (average PCI score of 41.1%), 153 ESS monitored and 317 providers briefed, and 2 out of 3 PPL assessed are at high risk in Oicha, with no infected PPL among the confirmed cases of the day.
  Of the 58 EDS alerts received and 44 postponements from the previous day, 55 EDS were carried out and 58 bodies swabed, without any collection failures; 47 EDS were postponed to the following day.

- **Bas-Uélé :** the providers of 7 ESS were monitored in 4 ZS (Buta 2/2, Koteli 1/1, Viadana 1/1 and Ganga 3/3), and 25 providers and 4 hygienists were briefed (100%) on the wearing of PPE, waste management and sorting; the DLMs are functional at the Koteli HGR (3/3), at the Rive gauche CS (1/1) and at the Rubi Lisanga CH (3/3), only the sorting of the Rive gauche CS is functional and the Kavio CS obtained an PCI score of 17%; no EDS alert was notified and 1 household and 1 ESS remain to be decontaminated in Dakwa.

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2.4.2. Challenges

- **In Ituri,** 8 out of 35 rings remain to be opened, 13 out of 58 households have not been decontaminated, 9 EDS have failed and 14 out of 79 bodies (17.7%) could not be swabed.

In North **Kivu,** a significant gap in ring road openings persists in Katwa, household decontamination remains low in Katwa and Kyondo, none of the 10 decontaminated households received kits (0/10), and 47 blood donations are still being postponed until the following day because families are not ready for burial. In Tshopo, the shortage of infection prevention and control (IPC) supplies and equipment in health facilities and communities, as well as the lack of trained IPC personnel, persists. In Haut-Uélé, the Pawa treatment center lacks a perimeter fence, has only one latrine, and lacks cleaning equipment. In Bas-Uélé, 16 out of 20 priority health facilities do not have functional triage, none have mattresses, 17 out of 20 do not have water storage sufficient for 72 hours, and the reporting of community deaths remains low.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 114 new admissions were recorded compared to 101 discharges: 7 recoveries. Thus, 393 patients are hospitalized, representing an occupancy rate of 38%, including 45 critically ill patients; 16 patients were evacuated by ambulance, 374 out of 382 patients (98%) received hot meals and 76 children are under LNPE/SLM (11 at the CTE, 4 at the nursery and 61 in the community).

- **In North Kivu,** 50 new admissions were recorded compared to 62 discharges: 1 recovery (Masereka). Thus, 471 patients are hospitalized, including 2 critically ill patients, with an occupancy rate of 85.1% for Ebola Treatment Centers (ETCs) (349/410), 92.7% for suspected cases (178/192), and 78.4% for confirmed cases (171/218). 122 patients (26%) are being treated in community health centers (ESS) that are not ETCs. 11 ambulance runs were successfully completed (8 out of 14 ambulances used), and 177 confirmed cases, 194 suspected cases, 419 people with HIV/AIDS (PPIs), and 197 caregivers received hot meals.

- **In Haut-Uélé,** two deaths of confirmed cases were recorded at the Pawa Ebola Treatment Center (ETC), from which two non-cases were also discharged; 39 confirmed cases remain hospitalized. Admission and bed occupancy data for October 17th were not provided.

- **In Tshopo,** two new admissions of suspected cases were recorded at the Météo treatment center, compared to one discharge (escape), with no recoveries or deaths. Thus, 19 patients are hospitalized out of 36 beds, representing an occupancy rate of 52.8%; six confirmed cases of Bafwasende have been lost to follow-up. Since the beginning of the outbreak, 44 confirmed cases have received the experimental treatment (3 MBP134 and 41 Remdesivir) and 464 people have received post-exposure prophylaxis.

- **In Bas-Uélé,** no new confirmed cases have been recorded; 3 patients remain in isolation, without recovery no death reported.

2.5.2. Challenges

- **In Ituri,** the low capacity of some CTE/CT (Kilo, Lolwa and Biakato), the care of 45 critical patients, the imminent risk of running out of blood (6 bags remaining), the lack of food for 8 patients in the Mandima health zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (12/33) remain worrying.

- **In North Kivu,** the beds for suspected cases are 92.7% full (178/192), 26% of hospitalized patients (122/471) are being treated outside CTE/CT and the completeness of data on ambulances is only 50% (Musienene, Beni, Butembo and Kalunguta missing).

- **In Haut-Uélé,** communication with the CTEs remains difficult due to the lack of Starlink connection in all sites, some staff are demotivated due to lack of payment, nutritional inputs (MUAC, toises, F75, F100, Plumpy'Nut) are lacking and the supply of water and energy remains insufficient in some CTEs.

- **In Bas-Uélé,** the lack of an ambulance for transferring patients and of a standardized CTE, still under construction at the HGR of Buta, as well as the lack of computer kits, remain major operational challenges.

2.6. Vaccination

2.6.1. Main actions

- **In Ituri,** vaccination of PPLs with Ervebo and research activities on the effectiveness of the vaccine against MVE-BDBV continue; the cumulative number of vaccinated PPLs reaches 1,260 (515 women and 745 men, including 15 pregnant women and 30 breastfeeding women), with 26 non-severe AFIs and 2 severe AFIs notified.
- **In North Kivu,** 130 people were vaccinated on October 17, 2026 in the Beni health zone, bringing the total to 192 vaccinated out of a target of 7,105 (2.7%); 600 doses of Ervebo vaccine and 600 syringes were received.

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and the PPLs of AS Mabakanga were sensitized on vaccination against Ebola. **In Haut-Uélé,** the cumulative number of PPLs vaccinated by Ervebo reached 589: 470 in Isiro (AS Mayogo and Tely), 80 in Pawa and 39 in Wamba (AS Bolebole).

2.6.2. Challenges

- **In Ituri,** the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (1,120 vaccinated) for a provisional target of 1,000).
- **In Bas-Uélé,** the start-up of the UCCs is not effective, vaccines are not available in sufficient quantity and vaccination funds are still pending; in Tshopo, a discrepancy of 551 vaccinated persists between the Excel and DHIS2 databases and the Bafwasende health zone has not reported for four days; in North Kivu, the vaccination team lacks means of mobility (2 vehicles) and 3 small tents, and the vaccine transport devices (ARKTEK) remain to be confirmed with the WHO.

2.7. Continuity of Care

2.7.1. Main actions • In North Kivu,

the baseline assessment of the Oicha General Referral Hospital (HGR) revealed a triage system that did not comply with the security protocol and was only functional until 5 p.m., the absence of PPE, pedal bins and hygiene products, an unsecured Ebola circuit in the laboratory, deliveries of suspected cases carried out in the common room and the absence of a follow-up plan for TB and HIV patients lost to follow-up; at the Kyondo General Referral Hospital (HGR), the triage system is not compliant, 13 out of 33 maternity staff are not trained in osteoarthritis and prenatal care, only 25 out of 205 staff (12.2%) are trained in infection prevention and control (IPC) and 50 staff are exposed, including 5 in quarantine.

- **In Tshopo,** the assessment of the Central Prison CP found 121 cases of malnutrition in adults and 10 PLHIV being monitored, a shortage of antituberculosis drugs for a cohort of 16 patients, very poor waste management, the absence of a triage area and no staff trained or vaccinated against Ebola (0/10); recovery plans for the Alwaleed CS and the Ketele military PS have been developed and follow-up of recovered patients with complications continues at the Makiso-Kisangani General Referral Hospital.
- **In Haut-Uélé,** the integration of catch-up vaccination of zero-dose and under-vaccinated children and of surveillance in monitoring the distribution of LLINs was advocated in the 13 health zones, the training of 7 health zone managers on SIMR continues and 5 PPLs remain being treated in the CTEs (2 in Wamba, 2 in Pawa and 1 in Boma Mangbetu); the deficit of subsidies for free care persists in the 7 most affected health zones.
- **In Ituri,** 79 children received nutritional support from LNPE/SLM: 9 out of 9 at the CTE, 6 out of 6 at the nursery and 64 out of 64 in the community; AMMA, distribution of MILD and space emanators were launched and medicines for free care were distributed in the targeted ZS.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 21,327 home visits reached 72,460 people; 460 educational talks brought together 10,143 people, 72 mass awareness campaigns reached 4,508 people, 7 community debates reached 331 people, and 13 advocacy sessions were held with 40 religious and community leaders (Rwampara, Komanda, Fataki, and Mandima). A total of 354 community alerts, including 186 deaths, were reported; 61 out of 79 feedback requests (77.2%) and 9 out of 9 incidents (100%) were resolved within 48 hours. The CREC facilitated the pre-listing of 152 contacts, the decontamination of 15 out of 15 households, the completion of 32 out of 37 DHS (86.5%), the announcement of 12 out of 12 positive results, and the transfer of 33 out of 34 suspected cases to the Ebola Treatment Center (ETC). The completeness of CREC reports remains low (10/28 ZS, 36%).

- **In North Kivu,** 10,626 home visits reached 34,917 people, 106 educational talks reached 2,856 people, 57 mass communications reached 5,271 people, and 4 briefings reached 77 people; 372 community alerts were notified out of 650 expected (57.2%), 19 out of 26 feedback requests were received (73.1%), and all incidents were resolved within 48 hours. 1,266 out of 4,750 people (26.7%) were engaged in community actions. A meeting of CREC partners, a public forum in Manguredjipa, and a community dialogue with PoC providers and RECOs in Beni were organized.

- **In Tshopo,** 318 households received home visits (2,212 people), 2 educational talks brought together 125 people and 11 mass awareness sessions reached 11,305 people in Lubunga, Kabondo and Makiso-Kisangani, and 2 radio programs were broadcast; doctors in charge of private health services were sensitized and engaged on Ebola, SGI meetings in 5 health zones were supported and the discharge of a recovered patient from the Météo Ebola Treatment Center was accompanied by testimonies; 2 out of 2 feedback requests and 1 out of 1 refusal were resolved.

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2.9. Mental Health and Psychosocial Support (MHPSS)
2.9.1. Main actions

- **In Ituri,** 23 confirmed cases out of 44 (52.3%), 81 confirmed cases under follow-up out of 126 (64.3%), 48 new suspects out of 114 (42.1%) and 106 suspects under follow-up out of 221 (48.0%) benefited from SMSPS interventions.
  The teams supported 253 caregivers and 99 PPLs, followed up with 17 out of 23 (73.9%) healers in the community and 12 out of 12 children in daycare, supported 11 out of 16 separated and orphaned children's households, reintegrated 4 out of 7 healers, carried out 2 family reunifications and announced 36 results, including 23 positive ones; 56 children are being followed up.

- **In North Kivu,** 12 of the last 14 confirmed cases (85.7%), 212 confirmed cases under follow-up, 211 new suspected cases and 215 suspected cases under follow-up benefited from SMSPS interventions, with the 6 health zones that notified cases being covered (100%); the teams accompanied 81 households and 37 households of separated and orphaned children out of 40 (92.5%), supported 55 PPLs, followed up 4 out of 4 children in daycare and 20 out of 36 children (55.5%) in the community, followed up 7 out of 10 recovered patients (70.0%) in the community, reintegrated 11 recovered patients and announced 87 results, including 14 positive ones; 313 people (153 men, 115 women and 45 children) benefited from psychoeducation with the support of UNICEF.

- **In Haut-Uélé,** only Isiro and Pawa reported: 1 new confirmed case, 10 confirmed cases under follow-up, 4 new suspects and 14 suspects under follow-up benefited from SMSPS interventions; 36 caregivers and 40 PPLs were supported, 4 separated children were followed in the community, 2 EDS were supported and 13 results announced, including 1 positive; 5 non-cases were discharged from the Pawa CTE and a 30-month-old suspected child was admitted to the Isiro CTE.

2.9.2. Challenges

- **In Ituri,** only 14 of the 28 affected health zones are covered (50.0%), data from the Mandima health zone, which concentrates 51% of the cases, is still not coming in and only 15 of the last 41 confirmed cases (36.6%) have been taken care of; training of psychologists and APS/TPS is scheduled for October 18 and 19, 2026.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana...

2.10. Logistics
2.10.1. Main actions.

- Ensure the mobility of all field teams in the affected provinces; in North Kivu, construction work on the huts at PoC PK5 and the hut and passenger walkway at Beni airport continues with IOM support; in Haut-Uélé, a team went down to Wamba for the briefing of service providers and the activation of PoC PK18 and Bangwene, the maintenance of an ambulance in Boma Mangbetu was monitored, 2 coordination vehicles were supplied with fuel for the mission in Bolebole and response inputs were delivered to the PoE/PoC pillar; In Bas-Uélé, the construction of the CTE at the HGR in Buta (partitioning of rooms and toilets) and the rehabilitation of the DPS offices with the support of the WHO are continuing, the surveillance teams have been deployed in Ganga and Viadana, 2 circuits of hand washing devices have been installed at the PoC PK4 Left Bank with a month's supply of water (support from FHI360) and the inputs for the ZS in preparation are being packaged.

2.11. Security

2.12. Main actions.

- In Tshopo, community tensions and a refusal of swabs were reported in Bafwasende, where attacks and threats against rapid response teams are hindering investigations into cases 65 to 70. In Haut-Uélé, the Wamba sub-coordination was relocated to the Bolebole-Mambati resistance zone, and teams were denied access to the Gosene Revival Church (Isiro), where alerts had been reported; in Ituri, the Avakubi PoC remains non-functional due to insecurity

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)
2.13.1. Main actions

- **In Ituri,** 99 participants (46 women and 53 men), including RECO supervisors, RECOs, and street leaders, were trained or briefed on the PSEAH and signed the code of conduct in the Bunia health zone, where 8 vendors were also sensitized; the display of messages and the distribution of leaflets continue in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki, and Mandima. 15 out of 36 health zones (41.7%) are aware of the reporting mechanism.

- **In North Kivu,** 28 data entry agents were briefed on the PSEA/HS in Beni, where 47 community relays from the ECUMR teams and 487 community members were sensitized; awareness sessions were also held in Butembo, reaching 21 people, 133 community members, and 29
  PRECODESA in Katwa, 23 people in Nyiragongo (AS Rusayo), 18 people in Mutwanga, 7 service providers in Mabalako and 7 teachers in Kalunguta, and 19 members of the epidemic management committee met in Walikale; 3 partners (IMC, WAAR and DRDC) reported on their activities

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2.13.2. Challenges

- At the national level, only Ituri and North Kivu have submitted their PSEA activities of October 17; coverage of affected areas remains low (56 health zones out of 151 with PSEA activities, i.e. 37.1%), mapping of stakeholders is not controlled and some partners refuse to align, awareness and visibility materials are insufficient, the effectiveness of CBCMs is low and the pillar lacks computer kits and office supplies.

KEY MESSAGES

Over the past 24 hours, 450 out of 458 suspected cases (98.3%) have been investigated in the provinces that submitted their data. However, 8 suspected cases remain uninvestigated in Ituri (Rwampara Health Zone). Furthermore, the contact tracing rate has improved, rising from 79.1% the previous day to 80.0%, but remains below the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.
Maintain a level of vigilance regarding epidemiological surveillance, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care facilities with adequate capacity.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities , with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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